DIGITAL MASTERCLASS

Surgery Prep Masterclass

Your Complete Guide to Preparing for Surgery — Mind, Body & Recovery

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1

Medication & Supplement Review

What to stop, what to keep, and when

One of the most important steps you can take before surgery is reviewing every medication, supplement, and vitamin you take with your surgical team. Some common over-the-counter products can increase bleeding, interfere with anesthesia, or interact with post-surgical medications. This module walks you through exactly what to look for.

> **Dr. Oluwole's Tip**
>
> I always tell my patients: bring every single bottle — prescription, supplement, vitamin, herbal tea — to your pre-op appointment. You'd be surprised how many "harmless" supplements can cause serious bleeding during surgery.

Blood Thinners & NSAIDs

These medications affect how your blood clots. Your surgeon and anesthesiologist will give you specific stop dates, but here are general guidelines:

Medication	Typical Stop Time	Notes
Aspirin (low-dose 81mg)	7 days before	Unless cardiologist says continue
Ibuprofen (Advil, Motrin)	7 days before	Switch to Tylenol for pain
Naproxen (Aleve)	14 days before	Longer-acting NSAID
Eliquis (apixaban)	2–3 days before	Per prescribing physician

Xarelto (rivaroxaban)	2–3 days before	Per prescribing physician
Warfarin (Coumadin)	5 days before	May need bridging therapy
Plavix (clopidogrel)	5–7 days before	Do NOT stop without cardiologist approval

> **IMPORTANT**
>
> Never stop a blood thinner on your own. Always confirm with the prescribing physician — especially if you have a heart stent, atrial fibrillation, or history of blood clots.

GLP-1 Medications (Ozempic, Wegovy, Mounjaro)

If you take a GLP-1 receptor agonist for weight loss or diabetes, this is critically important. These medications slow gastric emptying, which means food can remain in your stomach much longer than normal. This creates a real aspiration risk (food or liquid entering your lungs) under anesthesia.

Current ASA (American Society of Anesthesiologists) guidance:

- **Weekly injections** (semaglutide/Ozempic, Wegovy): Skip the dose due in the **week before** surgery (effectively a 7-day hold)
- **Daily injections** (liraglutide/Saxenda): Hold the **day of surgery**
- **Tirzepatide** (Mounjaro/Zepbound): Hold for **7 days** before surgery
- If GI symptoms (nausea, vomiting, bloating) are present, consider a **longer hold** and discuss with your anesthesiologist
- Your anesthesiologist may recommend a **gastric ultrasound** on the day of surgery to check for residual stomach contents

> **FORGOT TO HOLD YOUR GLP-1?**

If you forgot to hold your GLP-1 medication, tell your anesthesiologist immediately. Surgery may still proceed if you have no GI symptoms, but your team needs to know so they can take extra precautions.

Dr. Oluwole's Tip

I am seeing more and more patients on GLP-1 medications. If you're on one, please don't feel embarrassed to tell us — we just need to know so we can keep you safe. A full stomach under anesthesia is one of the most dangerous situations we face.

Supplements & Herbals to Stop

The general rule: **stop all supplements and herbal products at least 7–14 days before surgery** unless your doctor tells you otherwise.

Supplement	Why Stop?	Stop Timing
Fish oil / Omega-3	Increases bleeding	7–14 days
Vitamin E (high dose)	Increases bleeding	7–14 days
Garlic supplements	Increases bleeding	7 days
Ginkgo biloba	Increases bleeding	7 days
Ginseng	Affects blood sugar & bleeding	7 days
Turmeric / Curcumin	Blood-thinning properties	7–14 days
St. John's Wort	Drug interactions with anesthesia	14 days
Kava	Liver toxicity, sedation interaction	14 days
Valerian root	Sedation interaction	14 days

Echinacea	Immune & liver concerns	7 days
Green tea extract (capsules)	Bleeding risk at high doses	7 days

Medications to Keep Taking

Most chronic medications should be continued. Common ones to **keep taking** (with a small sip of water on surgery morning):

- **Blood pressure medications** — usually continue (except ACE inhibitors and ARBs — ask your surgeon)
- **Heart medications** (beta blockers, statins) — continue
- **Thyroid medication** — continue
- **Anti-seizure medications** — continue
- **Asthma inhalers** — continue (bring to hospital)
- **Antidepressants / Anti-anxiety** — usually continue (discuss MAOIs (a type of older antidepressant, such as phenelzine or tranylcypromine) specifically)

> **What About Metformin?**
>
> Metformin for diabetes is usually held the morning of surgery and for 48 hours after, especially if you're receiving IV contrast dye. Your surgeon will confirm.

☑ Medication Review Checklist

- [ ] Made a list of ALL medications, supplements, vitamins, and herbals I take
- [ ] Confirmed blood thinner stop date with prescribing doctor
- [ ] Informed surgeon about GLP-1 medication (if applicable)
- [ ] Stopped all supplements and herbals 7–14 days before surgery

- [ ] Know which medications to take morning of surgery with a small sip of water
- [ ] Brought medication list to pre-op appointment

2 Nutrition & Fasting Guidelines

Fuel your body for healing — and the truth about fasting

What you eat in the weeks before surgery — and what you eat (or don't eat) the night before — directly affects your healing, your energy levels, and even your infection risk. Let's clear up one of the biggest myths in surgery prep first.

The Midnight Fasting Myth

You've probably heard: *"Nothing to eat or drink after midnight."* For decades, this was standard. But modern evidence tells a different story.

Current ASA 2023 fasting guidelines:

What	Minimum Fasting Time	Examples
Clear liquids	2 hours	Water, apple juice (no pulp), black coffee, tea, Gatorade, Pedialyte
Breast milk	4 hours	For infants
Light meal / non-human milk	6 hours	Toast, crackers, formula
Full meal (fatty/fried)	8+ hours	Steak, fried foods, heavy meals

Dr. Oluwole's Tip

Here's what many patients don't realize: being *dehydrated* before surgery is actually harmful. Drinking clear liquids up to 2 hours before arrival can improve your comfort, reduce nausea, stabilize your blood pressure, and make your IV easier to start. Don't suffer through unnecessary thirst. Follow your surgeon's specific instructions, but know the science supports hydration.

Carbohydrate Loading

ERAS (Enhanced Recovery After Surgery) protocols recommend drinking a clear carbohydrate drink 2–3 hours before surgery. This helps your body manage blood sugar more effectively, preserves muscle, and helps you feel better after surgery.

- **The night before:** 800mL of a clear carbohydrate drink (like ClearFast or Gatorade)
- **Morning of surgery:** 400mL, finished at least 2 hours before arrival
- *Note: Diabetic patients — discuss carb loading with your surgical team first*

Protein Targets for Healing

Your body needs protein to repair tissues, fight infection, and rebuild strength. In the 2–4 weeks before surgery, aim for:

- **1.2–1.5 grams of protein per kilogram of body weight per day**
- For a 150-lb (68 kg) person: **82–102 grams of protein daily**
- Spread intake across 4–5 meals/snacks

High-protein food sources:

Food	Protein (approx.)
Chicken breast (4 oz)	35g
Salmon (4 oz)	25g
Greek yogurt (1 cup)	17–20g
Eggs (2 large)	12g
Lentils (1 cup cooked)	18g
Cottage cheese (1 cup)	28g
Whey protein shake	20–30g

Tofu, firm (1/2 cup)	10g

Healing Nutrients

- **Vitamin C** (citrus, strawberries, bell peppers) — wound healing, collagen formation
- **Zinc** (meat, shellfish, pumpkin seeds) — immune function, tissue repair
- **Vitamin A** (sweet potatoes, carrots, leafy greens) — cell growth, immune support
- **Iron** (red meat, spinach, lentils) — oxygen transport, energy
- **Vitamin D** (sunlight, fortified foods, supplements) — bone healing, immune function

Foods to Avoid Before Surgery

- **Alcohol** — stop at least 48 hours before (ideally 1–2 weeks); affects bleeding, liver function, and anesthesia
- **Excessive sodium** — contributes to swelling and fluid retention
- **Processed sugar** — impairs immune function and wound healing
- **Fried and greasy foods** — harder to digest, increase nausea risk

☑ Nutrition Checklist

- [ ] Increased protein intake to 1.2–1.5 g/kg daily
- [ ] Added healing nutrients (vitamin C, zinc, vitamin A) to my diet
- [ ] Stopped alcohol at least 48 hours before surgery
- [ ] Understand my fasting instructions (clear liquids 2 hrs, solids 6 hrs)
- [ ] Purchased carbohydrate loading drink (if recommended)

3 Prehabilitation

Train your body before surgery for faster recovery

Prehabilitation — "prehab" — is one of the most evidence-backed strategies to improve surgical outcomes. Patients who exercise before surgery recover faster, have fewer complications, shorter hospital stays, and less pain. Think of it like training for an athletic event — surgery is the event, and your body needs to be ready.

> **Dr. Oluwole's Tip**
>
> I tell my patients: surgery is a stress test for your body. The stronger you go into it, the stronger you come out. Even 2–3 weeks of consistent exercise makes a measurable difference.

Aerobic Exercise

Goal: 30 minutes, 3–5 times per week

Aerobic exercise improves your cardiovascular fitness, lung capacity, and circulation — all of which matter under anesthesia and during recovery.

- **Beginner:** Walking (brisk pace), stationary bike, water aerobics
- **Intermediate:** Jogging, cycling, swimming, elliptical
- **Advanced:** Running, HIIT intervals, rowing

Always start at your current fitness level and gradually increase. If you haven't exercised recently, walking is the single best exercise you can do.

Strength Training

Goal: 2–3 sessions per week

Muscle mass protects you during recovery. The more muscle you have going into surgery, the less strength you'll lose during your recovery period.

- **Upper body:** Wall push-ups, resistance band rows, overhead press (light weights)

- **Lower body:** Squats (bodyweight or light), lunges, step-ups, calf raises
- **Core:** Planks, bird-dogs, bridges — these support your spine and help with post-op mobility

Breathing Exercises & Incentive Spirometry

After surgery, your breathing will be shallower due to pain, sedation, and reduced mobility. Practicing deep breathing now trains your lungs and reduces your risk of pneumonia.

- **Incentive spirometer:** Use 10 times every hour while awake. Inhale slowly, hold 3–5 seconds, exhale. (Your surgeon may give you one at your pre-op visit; they're also ~$10 at any pharmacy.)
- **Diaphragmatic breathing:** Place hand on belly. Inhale through nose for 4 counts (belly rises), hold 4 counts, exhale through pursed lips for 6 counts. Repeat 10 times, 3x daily.
- **Coughing exercise:** Practice splinted coughing (hold a pillow against your incision area). This will be important post-op.

Sample Weekly Prehab Schedule

Day	Beginner	Intermediate	Advanced
Mon	Walk 20 min	Walk/jog 30 min	Run 30 min + weights
Tue	Chair exercises 15 min	Light weights 20 min	Strength training 30 min
Wed	Walk 20 min	Swim/cycle 30 min	HIIT 25 min
Thu	Rest + breathing exercises	Rest + breathing exercises	Active recovery + yoga
Fri	Walk 25 min	Walk/jog 30 min	Run 30 min + weights

Sat	Chair exercises 15 min	Light weights 20 min	Strength training 30 min
Sun	Gentle walk + stretching	Gentle walk + stretching	Rest + stretching

Every level: practice breathing exercises (incentive spirometry or diaphragmatic breathing) 3 times daily.

☑ Prehab Checklist

- [ ] Started aerobic exercise routine (walking counts!)
- [ ] Incorporated strength training 2–3x per week
- [ ] Practicing breathing exercises daily
- [ ] Purchased or received an incentive spirometer

4 Mental Preparation

Managing anxiety and setting expectations

Feeling nervous before surgery is completely normal. In fact, research shows that up to 80% of patients experience significant anxiety before their procedure. The good news? Mental preparation is a skill you can practice — and it genuinely improves outcomes. Patients who feel mentally prepared report less pain, need less medication, and recover faster.

> **Dr. Oluwole's Tip**
>
> I have been on both sides — as the anesthesiologist putting patients to sleep and as someone who understands what it feels like to trust your life to a team of people you may have just met. Your feelings are valid. What I can tell you is this: we have trained for years for exactly this moment. Our entire job is keeping you safe.

Managing Surgical Anxiety

Evidence-based strategies that work:

- **Education reduces fear.** The fact that you're reading this masterclass means you're already doing something powerful. Fear of the unknown is the biggest driver of surgical anxiety.
- **Name your specific fears.** "I'm scared of surgery" is harder to manage than "I'm worried about waking up during surgery" (which we can address directly — awareness under anesthesia is extremely rare, approximately 1 in 19,000 cases).
- **Limit Dr. Google.** After a certain point, online research increases anxiety rather than reducing it. You have this guide. Trust your surgical team.
- **Talk to someone.** A therapist, a friend, a family member, or our mental wellness counselor Stacey J. Floyd, MA, LPC.
- **Physical activity.** Exercise is one of the most effective anti-anxiety interventions. (See Module 3.)

- **Sleep hygiene.** Anxiety worsens with poor sleep. Maintain a consistent schedule, limit screens before bed, and avoid caffeine after noon.

Guided Visualization Exercise

This 7-step visualization can be done daily in the week before surgery. Find a quiet place, close your eyes, and walk through each step for 1–2 minutes:

1. **Arriving at the hospital:** Picture yourself walking in calmly, checking in, feeling prepared.
2. **Pre-op area:** You're in a warm gown, your IV is placed smoothly, your nurse is kind.
3. **Meeting your anesthesiologist:** They explain the plan. You feel heard and safe.
4. **Going to sleep:** You take a deep breath, the medication feels warm, and you drift peacefully.
5. **Waking up:** You open your eyes. It's over. You feel relieved. The team is there.
6. **Recovery room:** You're resting comfortably. Pain is managed. Your loved one is nearby.
7. **Going home:** You're in your own bed, healing, getting stronger each day.

> **Dr. Oluwole's Tip**
>
> Visualization isn't "woo-woo" — it's a tool used by elite athletes, military personnel, and surgeons. When you mentally rehearse a positive outcome, your nervous system responds as if it's already happening. This reduces cortisol (stress hormone) and primes your body for recovery.

10 Questions to Ask Your Surgeon

1. What are the specific risks of my procedure?
2. How many times have you performed this surgery?
3. What is the expected recovery timeline?
4. What are the signs of complications I should watch for?
5. How will my pain be managed after surgery?

6. When can I return to work / driving / exercise?
7. Are there any alternatives to surgery I should consider?
8. What should I do to prepare in the weeks before?
9. What medications should I stop or continue?
10. Who do I call if I have a problem after discharge?

8 Questions to Ask Your Anesthesiologist

1. What type of anesthesia will I have?
2. How will my nausea be prevented? (Especially if you have a history of it.)
3. I take [medication/supplement] — does that affect my anesthesia?
4. Will I have a breathing tube?
5. How will my pain be controlled during and after surgery?
6. What are the risks of anesthesia for me specifically, given my health history?
7. How long will it take to fully wake up?
8. Is there anything I can do to reduce my anesthesia risks?

Setting Realistic Recovery Expectations

One of the biggest sources of post-op frustration is mismatched expectations. Here's what to realistically expect:

- **You will not feel like yourself immediately.** Fatigue, brain fog, and emotional swings are normal for 1–4 weeks.
- **Recovery is not linear.** You'll have good days and setbacks. Both are normal.
- **Pain does not mean something is wrong.** Surgical pain is expected. Uncontrolled, worsening pain IS a concern.
- **Asking for help is strength, not weakness.** You will need a caregiver. Plan for it now.
- **Social media recovery timelines are misleading.** No one posts their worst days. Healing takes time.

☑ Mental Prep Checklist

- [ ] Identified and named my specific fears about surgery
- [ ] Practiced the guided visualization exercise at least once
- [ ] Prepared my list of questions for the surgeon
- [ ] Prepared my list of questions for the anesthesiologist
- [ ] Talked to someone about how I'm feeling
- [ ] Set realistic expectations for my recovery timeline

Dr. Oluwole's Tip

Want personalized, one-on-one guidance? Dr. Oluwole offers virtual Pre-Surgical Consultations where she reviews your complete health picture and builds a preparation plan specifically for your surgery. **Book your consultation →**

5

Day-of-Surgery Checklist

Everything you need the night before and morning of

The Night Before

- Shower with antibacterial soap (Hibiclens/chlorhexidine if instructed). Wash from neck to toes. Do not apply lotion, deodorant, or perfume after.
- Eat a light, easy-to-digest dinner (no heavy, fried, or fatty foods)
- Lay out comfortable, loose-fitting clothes for tomorrow (button/zip-front top is ideal)
- Pack your hospital bag (see packing list below)
- Set two alarms — you don't want to be rushed
- Charge your phone
- Remove nail polish (the anesthesia team monitors your nail beds for oxygen levels)
- Practice your breathing exercises one final time
- Do your visualization exercise (Module 4)

What to Pack

💼 Hospital Bag Checklist

- [ ] Photo ID and insurance cards
- [ ] List of all medications with dosages
- [ ] Advance directive / living will (if applicable)
- [ ] Comfortable, loose-fitting going-home outfit
- [ ] Slip-on shoes (no laces — you won't want to bend over)
- [ ] Phone charger

- [ ] CPAP machine (if you use one for sleep apnea)
- [ ] Glasses / contacts case (remove contacts before surgery)
- [ ] Comfort item (small pillow for seatbelt on ride home)
- [ ] This masterclass (printed or on phone) for reference

Surgery Morning

☑ Morning-of Checklist

- [ ] Shower with antibacterial soap again (if instructed for 2nd wash)
- [ ] No makeup, lotion, deodorant, perfume, or hair products
- [ ] No jewelry, piercings, or hair accessories (leave all at home)
- [ ] Took approved morning medications with a small sip of water
- [ ] Finished carbohydrate drink 2 hours before arrival (if instructed)
- [ ] Confirmed ride home with a responsible adult
- [ ] Arrived on time (usually 1.5–2 hours before scheduled surgery)

What to Tell the Anesthesia Team at Check-In

When you meet your anesthesiologist (usually 30–60 minutes before surgery), tell them about:

- Any allergies (medications, latex, food — especially eggs and soy)
- Previous anesthesia problems (nausea, difficult airway, family history of malignant hyperthermia)
- When you last ate or drank (be honest — this is a safety issue)
- All medications taken today

- Loose or capped teeth, dental work
- Sleep apnea or CPAP use
- Any cold, cough, or illness in the past 2 weeks
- Pregnancy possibility

> **Dr. Oluwole's Tip**
>
> Please be completely honest with your anesthesia team. We are not there to judge you — we are there to keep you safe. If you ate something you weren't supposed to, tell us. If you took a supplement you forgot to stop, tell us. We can adjust our plan, but only if we know the truth.

6

Recovery Roadmap

Week-by-week expectations for your healing journey

Recovery is a journey, not a single moment. Understanding what to expect each week helps you recognize normal healing and catch anything that needs attention. Here is your general roadmap — your surgeon will provide specifics for your procedure.

Week 1: The Hardest Part

This is the most challenging week. Your body is in acute healing mode.

- **Pain:** Peaks at days 2–3, then gradually improves. Pain at the incision site is expected.
- **Fatigue:** Profound tiredness. Anesthesia can take 24–72 hours to fully clear. You may sleep 12–16 hours a day.
- **Swelling & bruising:** Usually worst at days 3–5 before improving
- **Appetite:** May be poor. Focus on fluids and small, frequent meals.
- **Mood:** Feeling emotional, weepy, or irritable is very common. Anesthesia, pain medications, and the stress of surgery all affect your mood.
- **Activity:** Short walks (even just to the bathroom and back) starting day 1. This is the single most important thing you can do to prevent blood clots and pneumonia.

> **Dr. Oluwole's Tip**
>
> Week 1 is not the time to be a hero. Rest, accept help, take your medications on schedule, and walk a little bit every day. That's it. Everything else can wait.

Week 2: Turning the Corner

- **Pain:** Significantly better. Many patients transition off prescription pain medication.

- **Energy:** Starting to return, but you'll still tire easily. Don't push it.
- **Incision:** Should be healing with no active drainage or spreading redness
- **Activity:** Longer walks. Light household tasks. No lifting over 10 lbs (typically).
- **Follow-up:** Most surgeons schedule a 1–2 week post-op visit

Weeks 3–4: Building Back

- **Pain:** Mostly managed with over-the-counter medication (acetaminophen, ibuprofen if cleared)
- **Energy:** Noticeably improving. Still need more rest than normal.
- **Activity:** Gradually increasing. Walking 20–30 minutes. Some patients return to desk work.
- **Mood:** Stabilizing, though some patients experience a "post-op dip" around week 3–4 when initial relief wears off and frustration with limitations sets in

Normal vs. Concerning Symptoms

Normal	Concerning — Call Your Surgeon
Mild to moderate pain (improving daily)	Pain getting worse after day 3–4
Bruising and mild swelling	Increasing redness, warmth, or streaking from incision
Small amount of clear/pinkish drainage	Thick, cloudy, or foul-smelling drainage
Low-grade temperature (up to 100.3°F)	Fever above 100.4°F (38°C)
Fatigue and sleepiness	Confusion, difficulty staying awake
Mild constipation	No bowel movement for 4+ days despite treatment

Emotional ups and downs	Persistent depression, hopelessness, or thoughts of self-harm

Pain Management: The Multimodal Approach

Modern pain management uses multiple tools working together rather than relying solely on opioids:

1. **Acetaminophen (Tylenol):** Foundation of post-op pain control. 1000mg every 6–8 hours (max 3000mg/day) (your provider may approve up to 4000mg/day if you have no liver conditions and do not drink alcohol regularly). Take on a schedule, not "as needed."
2. **NSAIDs (ibuprofen/naproxen):** If cleared by your surgeon (not after certain surgeries). Alternate with acetaminophen.
3. **Ice therapy:** 20 minutes on, 20 minutes off. Reduces swelling and numbs pain.
4. **Nerve blocks / Regional anesthesia:** If used during surgery, may provide 12–72 hours of pain relief.
5. **Opioids (as rescue only):** Oxycodone, hydrocodone — use the lowest dose for the shortest time. (See Module 9 for details.)

7 When to Call Your Doctor vs. Go to the ER

Know the difference — it could save your life

After surgery, it can be hard to know what's normal and what's not. This guide helps you make the right call. When in doubt, **always call your surgeon's office first** — they have an after-hours line for a reason.

📞 Call Your Surgeon

- Fever above 100.4°F (38°C)
- Increasing pain not relieved by medication
- Wound changes: increased redness, warmth, swelling, or drainage
- Foul-smelling wound drainage
- Can't keep fluids or medications down for 12+ hours
- No bowel movement for 4+ days
- Persistent nausea or vomiting beyond 48 hours
- New numbness or tingling near the surgical site
- Medication concerns or side effects

🚨 Go to the ER / Call 911

- Chest pain or pressure
- Severe difficulty breathing or sudden shortness of breath
- Heavy bleeding that won't stop with pressure
- Signs of blood clot in leg (calf pain, swelling, warmth, redness in one leg)
- Sudden severe headache or vision changes
- Confusion or difficulty staying conscious
- Allergic reaction (hives, swelling, difficulty breathing)
- Fever above 103°F (39.4°C)
- Fainting or passing out

DVT vs. PE: Know the Difference

Blood clots are one of the most serious post-surgical complications. They can form in your leg veins (DVT) and travel to your lungs (PE). Knowing the signs can save your life.

DVT (Deep Vein Thrombosis)	PE (Pulmonary Embolism)

Pain or cramping in one calf or thigh	Sudden shortness of breath
Swelling in one leg (not both)	Sharp chest pain (worse with deep breath)
Warmth in the affected area	Rapid heart rate
Redness or discoloration	Coughing (may cough up blood)
Leg feels heavy or tender	Feeling lightheaded or faint

> **CRITICAL**
>
> **A PE is a medical emergency.** If you experience sudden shortness of breath or chest pain after surgery, call 911 immediately. Do not drive yourself.

Reduce your blood clot risk:

- Walk as soon and as often as your surgeon allows
- Do ankle pumps every hour while sitting or lying down
- Wear compression stockings as instructed
- Take blood thinners if prescribed (do not skip doses)
- Stay hydrated
- Avoid sitting or lying in one position for more than 2 hours

BONUS DEEP-DIVE MODULES

8

Understanding Anesthesia

Bonus Module — Types, the team, and what to expect

Anesthesia is one of the most common sources of fear before surgery — often because people don't fully understand what it involves. Let's demystify it.

Types of Anesthesia

Type	What It Does	Common Uses
Local	Numbs a small area. You're fully awake.	Skin biopsies, dental work, minor repairs
Regional — Nerve Block	Numbs a larger area (arm, leg, abdomen). You may be awake or lightly sedated.	Hand/arm surgery, knee arthroscopy
Spinal	Single injection in lower back. Numbs from waist down for 1–3 hours.	C-section, hip/knee replacement, hernia
Epidural	Catheter in lower back for continuous medication. Can be adjusted.	Labor & delivery, major abdominal surgery
Sedation (MAC)	"Twilight" — you're relaxed and drowsy but breathe on your own.	Colonoscopy, minor cosmetic procedures
General	Completely asleep. Breathing tube or mask. No awareness.	Major surgery, any procedure requiring complete stillness

The Anesthesia Team

- **Anesthesiologist (MD/DO):** Physician who plans your anesthetic, manages your vital signs, and handles emergencies. Board-certified after 4 years of specialized

training after medical school.

- **CRNA (Certified Registered Nurse Anesthetist):** Advanced practice nurse with specialized anesthesia training. May work independently or with an anesthesiologist depending on the facility.
- **Anesthesiologist Assistant (AA):** Works under the direct supervision of an anesthesiologist.

> **Dr. Oluwole's Tip**
>
> You have the right to ask who will be providing your anesthesia and what their qualifications are. A good anesthesia team will welcome your questions — it shows you're engaged in your care.

What Happens When You "Go Under"

1. You'll breathe oxygen through a mask for a few minutes ("pre-oxygenation")
2. IV medication is given — you'll feel warm and relaxed, then sleepy
3. You'll fall asleep in about 15–30 seconds
4. A breathing tube or airway device is placed (you won't feel this)
5. Throughout surgery, your anesthesiologist monitors: heart rate, blood pressure, oxygen, breathing, temperature, and depth of anesthesia

Waking Up

- As surgery ends, anesthesia is gradually reduced
- The breathing tube is removed (you may have a sore throat for 1–2 days)
- You'll wake up in the recovery room (PACU) — it may feel like only seconds have passed
- Feeling groggy, confused, cold, or emotional is completely normal
- Nausea can occur — tell your nurse immediately; they have medications ready

Common Anesthesia Side Effects

- **Sore throat** (from breathing tube) — 1–3 days, mild
- **Nausea/vomiting** — common, usually treatable (see Module 12)
- **Shivering** — common, resolves quickly with warming blankets
- **Grogginess/confusion** — 24–72 hours to fully clear
- **Muscle aches** — from medications used during anesthesia, resolve in 1–2 days
- **Hoarse voice** — usually resolves within a week

9

Pain Management Deep Dive

Bonus Module — The modern approach to surgical pain

Effective pain management is one of the most important factors in your recovery. The goal is not zero pain — some discomfort after surgery is inevitable and actually signals healing. The goal is *manageable* pain that allows you to move, breathe deeply, eat, and sleep.

The Multimodal Approach

Modern pain management layers multiple strategies together. Each one attacks pain through a different mechanism, so you get better relief with fewer side effects than relying on any single medication.

Step 1: Non-Opioid Foundation

- **Acetaminophen (Tylenol):** 1000mg every 6–8 hours. Take on a SCHEDULE, not "as needed." This is the single most important pain medication after surgery.
- **Ibuprofen (Advil/Motrin):** 400–600mg every 6–8 hours WITH FOOD. *Only if cleared by your surgeon.* Alternating with acetaminophen provides excellent coverage.
- **Naproxen (Aleve):** 220–440mg every 12 hours. Alternative to ibuprofen.

> **Alternating Schedule Example**
>
> 8 AM: Tylenol 1000mg → 11 AM: Ibuprofen 400mg → 2 PM: Tylenol 1000mg → 5 PM: Ibuprofen 400mg → 8 PM: Tylenol 1000mg → 11 PM: Ibuprofen 400mg. This provides near-continuous pain relief.

Step 2: Adjunct Therapies

- **Ice therapy:** 20 minutes on, 20 off. Wrap ice pack in a thin towel — never directly on skin or incision.
- **Elevation:** Keep surgical area elevated above heart level when possible
- **Nerve blocks:** If placed during surgery, may provide 12–72 hours of excellent pain relief. When the block wears off, start your oral medications BEFORE the pain

arrives.

- **Gabapentin/pregabalin:** Sometimes prescribed to reduce nerve pain and opioid needs

Step 3: Opioids (Rescue Only)

Opioids (oxycodone, hydrocodone, tramadol) are reserved for breakthrough pain that non-opioid medications can't control.

- Use the **lowest effective dose**
- Most patients need opioids for **3–5 days or less**
- Take with a stool softener (constipation is almost guaranteed — see Module 13)
- Do NOT drive, operate machinery, or make important decisions while taking opioids
- Do NOT mix with alcohol, benzodiazepines (Xanax, Valium), or sleep aids

Opioid Weaning Schedule

Don't stop opioids abruptly if you've been taking them for more than 3–5 days. A gradual taper prevents withdrawal:

- Days 1–3: Take as prescribed for breakthrough pain
- Days 4–5: Reduce to half dose, use only at night or for activity
- Days 5–7: Switch to acetaminophen + ibuprofen only
- If you're still needing opioids after 7–10 days, call your surgeon

Safe Storage & Disposal of Opioids

- **Store** in a locked location, away from children, teens, and visitors
- **Count** your pills — know how many you have
- **Never share** your prescription with anyone
- **Dispose** of unused pills at a pharmacy take-back program or mix with coffee grounds/cat litter in a sealed bag and throw away
- The DEA holds National Prescription Take-Back events twice a year

OPIOID SAFETY

Signs of opioid overdose: extreme drowsiness, slow/shallow breathing, blue lips or fingertips, unresponsiveness. If you see these signs in anyone, call 911 immediately and administer naloxone (Narcan) if available.

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Preventing Infection

Bonus Module — Evidence-based strategies to protect your incision

Surgical site infections (SSIs) occur in about 2–5% of surgeries. The good news: many are preventable with simple, evidence-based steps that start before you even get to the hospital.

Pre-Op Showering Protocol

- **Night before AND morning of surgery:** Shower with chlorhexidine (Hibiclens) soap if instructed. Wash from the neck down. Let the soap sit on your skin for 2 minutes before rinsing.
- Do NOT shave the surgical area. If hair removal is needed, the surgical team will use electric clippers (not razors — razors cause micro-cuts that harbor bacteria).
- Do NOT apply lotion, deodorant, perfume, or powder after your pre-op shower
- Use clean towels and put on clean clothes

Preventing Staph Bacteria Before Surgery

Some surgeons prescribe a preventive nose ointment protocol (prescription antibiotic mupirocin ointment inside both nostrils, 2x daily for 5 days before surgery) to eliminate Staph bacteria. This is especially common before:

- Joint replacement surgery
- Cardiac surgery
- Spine surgery with hardware
- Any implant-based procedure

Blood Sugar Control

Elevated blood sugar significantly increases infection risk — even in patients without diabetes. If you have diabetes:

- Optimize your blood sugar control in the weeks before surgery
- Target A1C below 8% (ideally below 7%) before elective surgery

- Your surgical team will monitor blood sugar closely during and after surgery

Post-Op Wound Care

- **Keep the dressing clean and dry** for the first 24–48 hours (or as instructed)
- **Wash hands** thoroughly before and after touching near your incision
- **Do not submerge** your incision in water (no baths, pools, hot tubs) until fully healed (usually 2–4 weeks)
- **Gentle shower** is usually okay after 48 hours — let water run over the incision, pat dry. Do not scrub.
- **Do not pick at** scabs, Steri-Strips, or surgical glue — let them fall off naturally

Warning Signs of Infection

- Increasing redness spreading from the incision
- Warmth and swelling that worsens after day 3–5
- Pus or thick, cloudy, or foul-smelling drainage
- Fever above 100.4°F
- Red streaks extending from the wound
- The wound opening up (dehiscence)

> **Dr. Oluwole's Tip**
>
> The single best thing you can do to prevent infection? Wash your hands. Every time. Before and after touching your incision area. It sounds simple because it is — and it works.

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The Recovery Room (PACU)

Bonus Module — What to expect when you wake up

The PACU (Post-Anesthesia Care Unit) is where you'll wake up after surgery. Knowing what to expect removes one more source of anxiety.

What PACU Looks Like

The recovery room is usually a large, open area with curtained bays. It's brightly lit, and you'll hear monitors beeping. There will be nurses nearby at all times. It can feel disorienting at first, and that's completely normal.

What's Connected to You

- **IV line:** In your hand or arm for fluids and medications
- **Pulse oximeter:** Small clip on your finger measuring oxygen levels
- **Blood pressure cuff:** Will inflate automatically every few minutes
- **Heart monitor:** Small stickers on your chest with wires (no pain)
- **Oxygen:** Nasal cannula (small tubes in your nose) or face mask
- **Urinary catheter:** For longer surgeries (you may or may not have one)
- **Surgical drain:** Small tube near your incision (depends on surgery type)

The Waking-Up Process

You may feel:

- **Confused or disoriented** — "Where am I? Is it over?" This is extremely common.
- **Cold** — Operating rooms are kept cool. Warm blankets will be provided.
- **Emotional** — Crying, laughing, or feeling anxious upon waking is normal. Anesthesia affects the brain's emotional centers.
- **Thirsty** — Your nurse will start with ice chips, then small sips of water.
- **Nauseated** — Tell your nurse immediately. Anti-nausea medication works fast.

- **Pain** — Your nurse will ask your pain level (0–10). Tell them honestly so they can treat it.

How Long You'll Stay

- **Outpatient surgery:** 1–3 hours in PACU before discharge home
- **Inpatient surgery:** 1–2 hours in PACU, then transferred to your hospital room

Discharge Criteria

Before you can go home, the nursing team will confirm:

- You're awake, alert, and oriented
- Pain is controlled
- Nausea is managed
- You can keep fluids down
- Vital signs are stable
- You have a responsible adult to drive you home and stay with you
- You've urinated (sometimes required, depends on procedure)

> **Dr. Oluwole's Tip**
>
> Your caregiver is just as important as your surgeon. Make sure the person taking you home understands your discharge instructions, knows your medication schedule, and has the surgeon's after-hours number. They are your safety net for the first 24 hours.

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Nausea & Vomiting

Bonus Module — Prevention and treatment strategies

Post-operative nausea and vomiting (PONV) affects 30–50% of surgical patients and is one of the most common complaints after surgery — often rated by patients as worse than pain. The good news: there are excellent prevention strategies.

Risk Factors for PONV

You are at higher risk if you have:

- Female sex (2–3x higher risk)
- History of PONV or motion sickness
- Non-smoking status
- Younger age
- Use of opioids after surgery
- Certain surgery types (gynecologic, abdominal, ear, eye, laparoscopic)
- Longer anesthesia time

> **Dr. Oluwole's Tip**
>
> Tell your anesthesiologist if you've had nausea after surgery before, if you get motion sickness, or if you're generally prone to nausea. We have a multi-drug prevention protocol that we can tailor specifically to your risk level. The more risk factors you have, the more anti-nausea medications we give during surgery.

Prevention Strategies

During surgery (your anesthesiologist will manage these):

- Ondansetron (Zofran) — most common anti-nausea medication
- Dexamethasone — steroid with anti-nausea properties
- Scopolamine patch (behind the ear) — for high-risk patients

- Minimizing inhaled anesthetics when possible
- Adequate IV hydration

At Home

- **Eat small, bland meals:** Crackers, toast, rice, bananas, applesauce
- **Stay hydrated:** Small sips frequently rather than large amounts at once
- **Ginger:** Ginger tea, ginger chews, or ginger ale (real ginger, not just flavoring) — evidence supports its anti-nausea effect
- **Peppermint:** Peppermint tea or peppermint aromatherapy (inhaling peppermint oil)
- **Fresh air and cool compresses:** Open a window, use a cool cloth on forehead
- **Avoid strong smells:** Cooking odors, perfumes, cleaning products
- **Acupressure:** Press the P6 point (inside of wrist, 3 finger-widths below wrist crease) for 2–3 minutes. Sea-Band wristbands use this principle.

When to Call Your Doctor

- Can't keep any fluids down for 12+ hours
- Can't take your medications due to vomiting
- Signs of dehydration (dark urine, dizziness, dry mouth)
- Vomiting blood or dark "coffee ground" material

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Constipation After Surgery

Bonus Module — Why it happens and how to prevent it

Post-operative constipation is one of the most underestimated and uncomfortable recovery challenges. Nearly every surgical patient experiences it to some degree. Let's talk about why it happens and how to stay ahead of it.

Why It Happens

- **Opioid medications:** The #1 cause. Opioids slow your entire digestive system.
- **Anesthesia:** General anesthesia temporarily slows gut motility
- **Reduced activity:** Moving less = slower digestion
- **Dehydration:** Not drinking enough water post-op
- **Dietary changes:** Eating less, eating differently, or NPO (nothing by mouth) period
- **Iron supplements:** If prescribed, iron is notoriously constipating

Prevention Protocol

Start BEFORE surgery, not after you're already miserable:

- **Docusate sodium (Colace):** 100mg twice daily — this is a stool softener, not a stimulant. Start the day before surgery. *Must-have if taking opioids.*
- **Senna (Senokot):** 1–2 tablets at bedtime — this is a gentle stimulant laxative. Add if docusate alone isn't enough by day 2 post-op.
- **Miralax (polyethylene glycol):** 17g (1 capful) in 8 oz water once daily — osmotic laxative, safe for daily use

> **Dr. Oluwole's Tip**
>
> My standard advice: if you are prescribed an opioid, you should also be taking a stool softener. Full stop. Don't wait until you're uncomfortable — start it with your first dose of pain medication. Prevention is so much easier than treatment.

High-Fiber Foods to Add When Eating Normally

- Prunes or prune juice (the classic for a reason)
- Pears, apples (with skin), berries
- Oatmeal
- Beans and lentils
- Broccoli, spinach, sweet potatoes
- Whole grain bread
- Ground flaxseed (1–2 tablespoons in yogurt or smoothie)

Hydration

Aim for at least **64 ounces (8 cups) of water daily**. Warm liquids (herbal tea, warm water with lemon) can be especially helpful for stimulating bowel activity.

When to Escalate

- No bowel movement for **3–4 days** despite stool softeners and laxatives → call your surgeon's office
- Severe abdominal pain, bloating, or distension → call your surgeon
- Nausea/vomiting with inability to pass gas → could indicate bowel obstruction → seek urgent care

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Caregiver's Guide

Bonus Module — For the person helping you recover

To the caregiver: Thank you. You are an essential part of this recovery, and your role matters more than you might realize. This module is specifically for you.

Before Surgery: Home Preparation

☑ Home Prep Checklist (For Caregivers)

- [ ] Move frequently used items to counter/waist height (no reaching or bending)
- [ ] Clear pathways of tripping hazards (rugs, cords, clutter)
- [ ] Set up a recovery station: bed or recliner with pillows, blankets, water, phone charger, remote, medications within reach
- [ ] Stock easy-to-prepare foods: soups, crackers, applesauce, protein shakes, yogurt
- [ ] Fill prescriptions BEFORE surgery day (pharmacy may be closed or busy after)
- [ ] Install grab bars or shower chair if needed (especially for orthopedic patients)
- [ ] Arrange childcare, pet care, or other household responsibilities for recovery period
- [ ] Have the surgeon's office number AND after-hours number saved in your phone

At the Hospital

- **Bring a notebook** — write down everything the surgical team tells you. The patient will NOT remember post-anesthesia instructions.

- **Ask questions** — when can they eat? shower? drive? lift? What medications, and when?
- **Take a photo** of the discharge instructions
- **Bring the car around** when the patient is ready — a wheelchair will take them to the entrance
- **Drive carefully** — every bump will be felt. Bring a pillow for between the seatbelt and incision.

Medication Management

For the first 24–48 hours, YOU should manage the medications:

- Set alarms on your phone for each medication
- Use a medication log: write down what was given and when
- Never let the patient double-dose — anesthesia causes memory fog
- Keep opioids in YOUR possession (the patient should not self-manage opioids while groggy)
- Watch for signs of over-sedation: difficulty waking, very slow breathing

Wound Care

- Wash your hands before and after helping with dressing changes
- Know what the incision looks like now (take a photo at discharge as a baseline)
- Watch for changes: increased redness, swelling, drainage, warmth, or odor
- Help the patient follow the surgeon's wound care instructions exactly

Emotional Support

- Post-surgical depression and mood swings are common — be patient
- The patient may be frustrated with their limitations — validate their feelings
- Encourage small goals: "Let's walk to the kitchen today"
- Limit visitors if the patient is overwhelmed

- Watch for concerning signs: persistent hopelessness, withdrawal, not eating, mentioning self-harm

Self-Care for Caregivers

> **Dr. Oluwole's Tip**
>
> Caregiver burnout is real. You cannot pour from an empty cup. Take breaks, accept help from others, eat well, and sleep when you can. Your well-being matters too — and a rested, healthy caregiver provides better care.

- Accept help when offered — let others bring meals, do laundry, or sit with the patient so you can rest
- Take 15–30 minutes daily for yourself (walk, read, shower in peace)
- Don't neglect your own medications or appointments
- It's okay to feel frustrated, scared, or overwhelmed — you're human

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This guide is educational and not a substitute for your physician's specific instructions. Always follow your surgeon's and anesthesiologist's personalized recommendations. Based on ASA (American Society of Anesthesiologists), ERAS (Enhanced Recovery After Surgery), and ACS (American College of Surgeons) evidence-based guidelines.